

TIMS GP Guide Spinal – detailed management guidance

GP Management					
Condition / Diagnosis	Symptom(s)	Recommended management	Imaging guidance	When to refer to TIMS	When to refer to secondary care
Mechanical spinal pain (cervical, thoracic or lumbar)	- Neck pain; can refer down towards shoulders or up into back of the head, worse with neck movements or sustained postures - Pain around the thoracic spine/mid back/ribs - Lower back pain, can refer into buttocks/thighs	Optimise analgesia with prescribed/over the counter meds: - Consider oral NSAIDs - Consider weak opioids (with or without paracetamol) for acute low back pain only if an NSAID is contraindicated, not tolerated or has been ineffective - Consider a short course of diazepam (2-5days) for those with acute low back pain and muscle spasm - Do not offer paracetamol alone - Do not routinely offer opioids for acute low back pain - Do not offer opioids for chronic low back pain NICE- pharmacological management of Sciatica NICE- non specific neck pain - Advice and re-assurance hurt does not equal harm - Encourage activity, including specific exercises and return to normal lifestyle - Encourage early return to work - Address expectations about investigations - Support the patient with self-management - Discourage the use of cervical collars or spinal support/brace as this may restrict range of movement and prolong symptoms. - Advise the patient not to drive if the range of movement in the neck is restricted - Discuss weight management, alcohol moderation and smoking cessation where appropriate (healthy conversations) - NHS- Live well - Pain management strategies including pacing, relaxation and lifestyle changes. - Reinforce/push message overwhelming majority of low back pain is normal and 92-96% will clear up within 6-12 weeks without any clinical intervention	Only when red flags/ serious cause to pain is suspected	Diagnostic uncertainty If patient has not responded to recommended management despite good compliance and understanding; pain is not improving with self- management with 6 weeks, or, if pain is intolerable refer earlier	Not indicated
Patient Information – Neck & Back: The Neck Book/ The Back Book, TIMS Management of neck pain TIMS Managing Thoracic Back Pain TIMS Management of Back pain Patient Information - Pain: https://mskr.info/musculoskeletal/how-does-pain-work/ log in mskr 10 facts every person should know about back pain Understanding pain in less than 5 minutes – The single most important thing you can do for your stress: Exercise videos NHS Fitness Studio					

Cervical radicular pain/ radiculopathy	Neck pain referring into arm: • Radicular pain - electric shock like, sharp, shooting, pain extends below elbow • Arm pain worse than neck pain • +/- P&N/numbness in the arm • +/- weakness in arm/loss of function Radicular pain- due to nerve irritation causing pain or PNS. Arm pain is severe, constant, difficult to find a comfortable position when acute. May be due to ischaemia and neural inflammation, not compression, causing sensitisation of the nerve. This will help patient understand if they have an MRI and no compression is identified. Dermatomal pattern in 1/3 of patients. May have normal hard neuro findings on assessment but restricted/painful nerve tension tests such as ULNT Radiculopathy-is not a pain condition but impaired nerve conduction- will have a loss of function causing numbness or weakness. They may describe PNS; electric shock type symptoms; sharp, shooting, odd sensations such as running water, numbness, weakness such as dropping things. Pain extends below elbow. Arm pain is worse than neck pain. Loss of function- weakness, numbness Pain may coexist with radiculopathy.	Safety net those patients at risk of myelopathy and provide patient with relevant patient information leaflet: Myelopathy • Address expectations about investigations – for suspected serious cause or surgical target • Majority of radicular pain will resolve without needing surgical intervention and surgery is for arm pain not for neck pain Management as per mechanical neck pain: Optimise analgesia with prescribed/over the counter meds: NICE- non specific neck pain • Offering oral analgesia to relieve symptoms • Considering offering amitriptyline, duloxetine, pregabalin or gabapentin • Consider offering a muscle relaxant • Advice and re-assurance hurt does not equal harm • Encourage activity, including specific exercises and return to normal lifestyle • Encourage early return to work • Address expectations about investigations • Support the patient with self-management • Discourage the use of cervical collars or spinal support/brace as this may restrict range of movement and prolong symptoms. • Advise the patient not to drive if the range of movement in the neck is restricted • Discuss weight management, alcohol moderation and smoking cessation where appropriate (healthy conversations) - NHS- Live well • Pain management strategies including pacing, relaxation, lifestyle changes, relaxation • Reinforce/push message majority will improve within 6-12 weeks without any clinical intervention Patient Information: TIMS Management of Neck Management of neck pain	Routine MRI for radicular symptoms not improving with conservative treatment without red flags Urgent MRI if suspected serious pathology causing symptoms or neuro deficit worsening (weakness <3/5)	Diagnostic uncertainty If patient has not responded to recommended management despite good compliance and understanding For assessment for suitability for further investigation/onward referral to secondary care If there is any weakness less than grade 3	Deteriorating neurology- refer to serious pathology pathways for myelopathy as appropriate Persistent radicular symptoms with surgical target identified on MRI (ie nerve root compression corresponding with clinical symptoms) as cause of symptoms and patient wishes to consider further intervention such as injection or surgery and is medically fit for surgery
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Lumbar radicular pain/ radiculopathy Lumbar stenosis	Back pain referring into leg: • Radiculopathy / radicular pain - electric shock like, sharp, shooting, pain extends below knee • Leg pain worse than back pain • +/- P&N/numbness in leg • +/- weakness in leg/loss of function • Can be referred to bilateral/unilateral buttocks in lumbar stenosis (see cervical radicular pain/radiculopathy for further details)	• Address expectations about investigations – for suspected serious cause or surgical target • Majority of radicular pain will resolve without needing surgical intervention and surgery is for leg pain not for back pain Safety net those patients at risk of CES and provide patient with CES patient information Management as per mechanical back pain: Optimise analgesia with prescribed/over the counter meds: • Consider oral NSAIDs • Consider weak opioids (with or without paracetamol) for acute low back pain only if an NSAID is contraindicated, not tolerated or has been ineffective • Consider a short course of diazepam (2-5days) for those with acute low back pain and muscle spasm • Do not offer paracetamol alone • Do not routinely offer opioids for acute low back pain • Do not offer opioids for chronic low back pain NICE- pharmacological management of Sciatica • Advice and re-assurance hurt does not equal harm • Encourage activity, including specific exercises and return to normal lifestyle • Encourage early return to work • Address expectations about investigations • Support the patient with self-management • Discourage the use of cervical collars or spinal support/brace as this may restrict range of movement and prolong symptoms. • Advise the patient not to drive if the range of movement in the neck is restricted • Discuss weight management, alcohol moderation and smoking cessation where appropriate (healthy conversations) - - NHS- Live well • Pain management strategies including pacing, relaxation, lifestyle changes, relaxation • Reinforce/push message overwhelming majority of low back pain is normal and 92-96% will clear up within 6-12 weeks without any clinical intervention • For those with a stenotic pattern of pain- buttock or leg pain, worse with standing/walking, eases on flexion/sitting, advice on flexion based exercises - TIMS Spinal Spinal Stenosis Patient Information: TIMS Management of Leg Pain linked to the Lower Back (Sciatica) What is sciatica? https://www.youtube.com/watch?v=HJvNBYOKf64	Routine MRI for radicular symptoms not improving with conservative treatment without red flags Routine MRI for suspected spinal stenosis and neurogenic claudication restricting walking distance Urgent MRI if suspected serious pathology causing symptoms or neuro deficit worsening (weakness <3/5)	Diagnostic uncertainty If patient has not responded to recommended management despite good compliance and understanding For assessment for suitability for further investigation/on ward referral to secondary care	Persistent radicular symptoms with surgical target identified on MRI (ie nerve root compression corresponding with clinical symptoms) as cause of symptoms and patient wishes to consider further intervention such as injection or surgery and is medically fit for surgery Moderate- severe stenotic changes reported on MRI scan which correlate with symptoms and would consider and is medically fit for surgery
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Whiplash associated disorder (WAD)	Once signs of serious neck/head injury have been excluded, diagnose whiplash injury based on the following features: • History of whiplash type injury related to the onset of current symptoms • Neck pain that may refer to the shoulder or arm. • Reduced range of neck movements. • Muscular spasm • Headache • Fatigue, dizziness, paraesthesia, or nausea (less common).	Advice and re-assurance: • Symptoms are a normal reaction to being hurt • Maintaining normal life activities is an important factor in getting better • Staying active is important in the recovery process • Voluntary restriction of activity may cause delayed recovery. • It is important to focus on improvements in function • Encourage return to normal activity and lifestyle • Encourage early return to work • Address expectations about investigations. • Support the patient with self-management. • Discourage the use of cervical collars as this may restrict range of movement and prolong symptoms • Advise the patient not to drive if the range of movement in the neck is restricted. • Discuss weight management and smoking cessation where appropriate (healthy conversations) – • NHS- Live well • Pain management strategies, including pacing, sleep, relaxation, lifestyle changes. Patient Information: Whiplash Injury Recovery booklet The Whiplash book	Routine MRI if radicular symptoms not improving with conservative treatment without red flags Urgent MRI if suspected serious pathology causing symptoms or neuro deficit worsening (weakness <3/5)	Diagnostic uncertainty If patient has not responded to recommended management despite good compliance and understanding	Not indicated for WAD – manage radicular symptoms or serious pathology as per guidance
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Coccyx pain/ coccydynia	The main symptom is pain and tenderness in the area just above the buttocks The pain may: • be dull and achy most of the time, with occasional sharp pains • be worse when sitting down, moving from sitting to standing, standing for long periods, having sex and going for a poo • make it very difficult to sleep and carry out everyday activities, such as driving or bending over	Advice and re-assurance: • Use a specially designed coccyx cushion – these can be bought online and from some shops; they help reduce the pressure on your tailbone while you're sitting down • Avoid prolonged sitting whenever possible – try to stand up and walk around regularly; leaning forward while seated may also help • Wear loose-fitting clothes – avoid clothing such as tight jeans or trousers that may put pressure on your tailbone • Apply warm and cold packs to your tailbone – warm packs include hot water bottles and microwaveable heating pads; cold packs are available as freezable gel-filled pads from pharmacies, or you can use a bag of frozen vegetables wrapped in a towel • Lying on your side with a pillow between your knees may be helpful at night • Adjust how you get in and out of bed – try to roll onto your side and push yourself up, rather than sitting up directly onto the coccyx • Try laxatives if the pain is worse when you are opening your bowels - many laxatives are available to buy from pharmacies and supermarkets without a prescription • Take over-the-counter painkillers- simple pain killers and non-steroidal anti-inflammatory (NSAID) painkillers that you can buy from shops and pharmacies, such as ibuprofen • Pain management strategies such as pacing, relaxation, sleep Ref: Coccydynia (tailbone pain) - Treatment - NHS (www.nhs.uk) Patient Information: TIMS Management of Coccydynia	Routine MRI if associated with a history of trauma or osteoporosis - lumbar spine, including sacrum /coccyx to exclude fracture in absence of trauma, include distal coccyx to exclude malignancy	Diagnostic uncertainty or to exclude serious pathology such as chordoma If patient has not responded to recommended management despite good compliance and understanding	For persistent severe pain despite appropriate conservative management, referral to spinal orthopaedics
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Sacroiliac Joint / buttock pain	Pain in unilateral/bilateral buttocks or around SIJ Pain can be referred to this region from the lumbar spine or hips or local to the area Can be associated with pregnancy, childbirth, pelvic pain or axial spondyloarthritis (AxSpA) Buttock pain can be a feature of spinal stenosis	- Advise on sitting - sit on both buttocks evenly - do not cross your legs or sit with your feet up to the side of the chair - Avoid low chairs and support the low back with a rolled-up towel. Sitting facing the back of a chair with one leg either side is often comfortable - Advice on getting in and out of bed/cars Plus: Management as per mechanical back pain: Optimise analgesia with prescribed/over the counter meds: - Providing adequate analgesia (an NSAID first –line or codeine with or without paracetamol if an NSAID is contraindicated or not tolerated). If the paraspinal muscles are in spasm, a short course of benzodiazepine such as diazepam can also be considered - Advice and re-assurance hurt does not equal harm - Encourage activity, including specific exercises and return to normal lifestyle - Encourage early return to work - Address expectations about investigations - Support the patient with self-management - Discourage the use of or spinal support/brace as this may restrict range of movement and prolong symptoms. - Discuss weight management, alcohol moderation and smoking cessation where appropriate (healthy conversations) - NHS- Live well - Pain management strategies including pacing, relaxation, lifestyle changes, relaxation - Reinforce/push message overwhelming majority of low back pain is normal and 92-96% will clear up within 6-12 weeks without any clinical intervention - For those with a stenotic pattern of pain- worse with standing/walking, eases on flexion/sitting, advice on flexion based exercises Patient Information TIMS Leg Pain linked to the Lower Back (Sciatica) TIMS Management of Spinal Stenosis when clear stenotic pattern- buttock or leg pain worse on walking/standing, better with flexion/sitting	X-ray for hip pathology MRI for lumbar stenosis/radicular pain	Diagnostic uncertainty If patient has not responded to recommended management despite good compliance and understanding	If HIP x-ray confirms significant osteoarthritis- follow hip OA pathway MRI confirms significant lumbar spinal stenosis/radiculopathy, refer to spinal orthopaedics The following should be referred to rheumatology service using e-RS: - Suspected AxSpA (axial spondyloarthritis) that requires diagnosis - New diagnoses of AxSpA or inflammatory patient struggling with the condition - Chronic AxSpA with more underlying comorbidities that may affect rehab potential
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